

Artifact 18 — Partnership Mapping

Strategy · Stage 4 · Go-to-Market Architecture

DOCUMENT DETAILS

Project	Home-Care-AI	Stage	Strategy → Stage 4 (Go-to-Market Architecture)
Date	2026-03-27	Methodology	Healthcare Value Chain mapping; Moat-Builder Score (SC + NE); Tier 1/2/3 prioritisation
Input	Artifact 12 (Startup Canvas — channels, capabilities, confirmed v1 infrastructure), Artifact 13 (SWOT — VI-1 EMR integration assumption, 12-month AlayaCare window, O2/O6 NDIS opportunity), Artifact 17 (Remediation Decision Record — MessageMedia/AWS SNS v1 confirmed, Google Maps APP 8 review pending)		
Regulatory	Australian Privacy Act 1988 (APP). HIPAA-grade security as design floor. APP 8 governs any partner involving cross-border data processing.		
Feeds into	Artifact 19 — Positioning Statement (mandatory input per CLAUDE.md Article IV Rule 5); agentic-logic-spec partner NFRs		

CLAUDE.md Article IV Rule 5: positioning-statement cannot run before partnership-mapping. Moat claims without confirmed partners are capped at 3/5 and flagged ■ Unvalidated. The **Moat Summary for Positioning** row at the end of §2 is the direct feed to Artifact 19.

Scope note: This map is adapted entirely for the Australian home care ecosystem. US-centric references (Epic, Medicare Advantage, CMS) are replaced by their Australian equivalents (AlayaCare, My Aged Care, ACCPA). The product is pre-revenue, pre-launch — most partnerships are aspirational. Tier classification reflects this honestly.

Table of Contents

Table of Contents	2
1. Partnership Ecosystem Map	4
Australian Home Care Value Chain	4
Partner Categories Summary	4
2. Partnership Prioritisation Matrix	5
Integration Partners	5
Validation Partners	6
Distribution Partners	7
Moat Summary for Positioning (Artifact 19 Direct Feed)	9
3. Value Proposition per Partner Type	10
For Integration Partners (AlayaCare — Primary Target)	10
For Validation Partners (ACCPA — Primary Target)	10
For Distribution Partners (My Aged Care / Services Australia — Strategic Target)	10
4. Top 3 Moat-Building Partnerships — Deep Dive	11
Deep Dive 1 — ACCPA (Aged & Community Care Providers Association) [NE][CV]	11
Deep Dive 2 — My Aged Care (MAC) / Services Australia [SC][NE]	11
Deep Dive 3 — AlayaCare (EMR API Integration) [SC]	12
5. Partnership Risks & Red Flags	15
6. Handshake Table — HS-STRAT-05	16
7. Action Items	17
Partnership Investment Summary (PAC Estimates → Artifact 20 inputs)	17

1. Partnership Ecosystem Map

Australian Home Care Value Chain



Partner Categories Summary

Category	Partners Mapped	Confirmed v1	Tier 1 (launch)	Tier 2 (6–12 mo)	Tier 3 (Year 2+)
Integration	8	3 (MessageMedia, AWS, Google Maps)	4	3	1
Validation	5	0	1	2	2
Distribution	5	0	2	2	1
Total	18	3	7	7	4

2. Partnership Prioritisation Matrix

Moat-Builder Score = SC Score (Switching Cost, 1–5) + NE Score (Network Effect, 1–5). Max 10.

Tier 1 (Launch): Score ≥ 6, Complexity Low–Medium, OR confirmed v1 infrastructure.

Tier 2 (6–12 months): Score 4–6, or strategically critical with High Complexity.

Tier 3 (Year 2+): Score < 4, aspirational, or conditional on beachhead validation.

Integration Partners

Partner	Moat Type	S C	N E	Tot al	Strategic Rationale	Complexity	Timeline
MessageMedia / AWS SNS ap-southeast-2 (AU SMS gateway) [SC]	Switching Cost (infrastructure lock-in)	2	1	3	v1 confirmed — resolves CRIT-01. Domestic processing, no APP 8 trigger. All external notifications routed through this layer. Channel Wrapper makes future migration easy but creates mild lock-in through integration depth.	Low — API-based, well-documented	v1 confirmed
AWS ap-southeast-2 (Lambda, DynamoDB, CloudTrail, S3 WORM) [SC]	Switching Cost (infrastructure lock-in + audit log immutability)	3	1	4	v1 confirmed — CloudTrail is the immutable audit log store (CLAUDE.md 7-year retention requirement). DynamoDB for SPP + availability index. S3 Object Lock for WORM audit storage. Migration to another cloud = re-architect the audit trail.	Low — standard managed services	v1 confirmed
Google Maps Distance Matrix API [SC]	Minimal commodity service	1	1	2	v1 confirmed — proximity computation (ACT-V-03). Only carer postcode transmitted. APP 8 review pending (SC-07 from Artifact 10). Low switching cost — a local geocoding library could replace it.	Low — single API call per match	v1 confirmed ■ SC-07 pending
AlayaCare (EMR API integration) [SC]	Switching Cost (workflow embed)	4	1	5	VI-1 assumption — established agencies use AlayaCare for rostering; an integration means coordinators never leave their existing workflow to use Home-Care-AI. Kills the "another app" objection. Resolves the single most-cited adoption risk in Artifact 12 W1. If AlayaCare offers exclusivity, it flips from partnership to threat (see §5).	High — FHIR/HL7 interface; App Marketplace listing process	Tier 2 — initiate after E1 validates standalone value
ShiftCare (SME home care scheduling) [SC]	Switching Cost (workflow embed)	3	1	4	Serves agencies in the 5–50 client range — smaller than our primary beachhead but strongly aligned with the O6 greenfield pivot path (new NDIS agencies with no existing EMR). Lower integration complexity than AlayaCare. VI-1 is not a barrier for agencies that use ShiftCare — they are already on a lightweight platform. Locked-gate risk near-zero — ShiftCare has no competing Vacant Visit Agent and no incentive to block. Elevated to Tier 1 parallel track in the event of AlayaCare denial (see §4 AlayaCare Contingency Map).	Medium — REST API, simpler than AlayaCare	Tier 1 ■ — parallel track; activate if AlayaCare denies or delays

Partner	Moat Type	S C	N E	Tot al	Strategic Rationale	Complexity	Timeline
HCP (Home Care Platform) [SC]	Switching Cost (workflow embed)	3	1	4	Second-largest AU home care platform after AlayaCare. Primarily NDIS-focused. Overlaps with O6 greenfield segment. Lower priority than AlayaCare for the beachhead, but relevant if NDIS expansion is validated.	Medium — REST API	Tier 2
Humanforce (aged care workforce management) [SC][NE]	Switching Cost + mild Network Effect (shared workforce pool across agencies)	2	2	4	Humanforce manages carer scheduling, availability, and credential tracking for large AU aged care providers. An integration would give Home-Care-AI real-time availability data (S-2) rather than relying on self-reported postcodes — solving the F-1 supply-side risk (carer availability reliability). Only valuable if Humanforce's client base overlaps with our beachhead agencies.	Medium	Tier 2
Mable / Careseekers (peer-to-peer carer marketplace) [NE]	Network Effect (carer supply pool)	1	3	4	Mable is Australia's largest peer-to-peer care marketplace (~20,000 carers). An integration or data-sharing agreement would give Home-Care-AI access to a broad pool of available carers beyond the agency's own workforce — directly addressing the EC-02 edge case (no eligible SPP candidate). Complex privacy considerations: Mable carers are independent contractors, not agency employees. APP consent model is different.	High — two-sided marketplace with different data governance	Tier 3 — requires NDIS segment validation first

Validation Partners

Partner	Moat Type	S C	N E	Tot al	Strategic Rationale	Complexity	Timeline
ACCPA (Aged & Community Care Providers Association) [CV][NE]	Clinical Validation + Network Effect (industry endorsement)	1	5	6	ACCPA is the peak body for 1,000+ community care providers in Australia, formed by the 2022 merger of ACSA and LASA. Membership covers the entire independent home care TAM. An ACCPA endorsement or co-publication operates as: (a) a trust signal that converts agency owner LOIs (GTM-1), (b) a distribution channel through ACCPA member communications and the Future of Aged Care conference, and (c) clinical validation evidence for the Agency Owner value proposition. Recruiting an ACCPA-connected clinical advisor now costs nothing.	Low — start with Advisory Board invitation; no commercial agreement required initially	Tier 1 — initiate at E1 start

Partner	Moat Type	S C	N E	Tot al	Strategic Rationale	Complexity	Timeline
Dementia Australia [CV]	Clinical Validation (high-P3 segment credibility)	1	3	4	Clients with P-3 = "known carers only" are disproportionately people living with dementia. A Dementia Australia endorsement or co-developed care continuity standard would: (a) validate the SPP familiarity threshold feature for the highest-risk client subpopulation, (b) unlock the dementia care segment within agencies, and (c) strengthen the DPIA case for P-3/P-4 data collection by demonstrating clinical justification.	Low-Medium — position product as care continuity enabler, not clinical tool	Tier 2
Aged Care Quality and Safety Commission (ACQSC) [CV]	Clinical Validation (regulatory alignment)	1	2	3	The ACQSC enforces the Aged Care Quality Standards (Standard 3: Personal Care and Clinical Care; Standard 7: Human Resources). Demonstrating that Home-Care-AI's E-3 gate and SPP improve compliance with Standard 3 (consistency of care) and Standard 7 (qualified, appropriate carers) creates a regulatory alignment narrative. Not a commercial partnership — an alignment document.	Low — submit product to ACQSC's Innovation Hub (AU Govt); no commercial terms	Tier 2
Bolton Clarke / Royal District Nursing Service (RDNS) [CV]	Clinical Validation (pilot data)	2	1	3	Bolton Clarke is Australia's largest independent not-for-profit home care provider (~100,000 clients). A pilot agreement would produce Real-World Evidence (RWE) at scale: vacancy fill rates, cancellation rates, client satisfaction scores. This data is the evidence base for Agency Owner LOIs (OKR-1 at scale) and for ACCPA co-publication. High-effort to initiate (large provider procurement cycle) but high-value output.	High — large provider procurement cycle, 6–12 months to first data	Tier 2 — initiate after beachhead (Angela/Tom) validation
NARI (National Ageing Research Institute, Royal Melbourne Hospital) [CV]	Clinical Validation (academic publication)	1	2	3	NARI is Australia's leading academic aged care research institute. Co-authoring a study on "care continuity and trust-based matching" with NARI would produce a peer-reviewed publication — the gold standard for enterprise buyer and payer credibility. 18–24 month timeline.	High — research partnership agreement, IRB equivalent (HREC), 18–24 months to publication	Tier 3 — Year 2 post-beachhead

Distribution Partners

Partner	Moat Type	S C	N E	Tot al	Strategic Rationale	Complexity	Timeline
ACCPA (member communications + conference) [NE]	Network Effect (industry distribution)	1	5	6	Listed above as validation partner — ACCPA plays a dual role. As a distribution partner, ACCPA's member newsletter (1,000+ organisations), the Future of Aged Care conference, and the ACCPA Innovation Awards are the lowest-CAC distribution channel for the independent agency segment. A conference demonstration at the right moment (post-E1 validation, post-reference customer) converts to Agency Owner LOIs at scale.	Low — sponsorship or speaker slot at industry events; no integration required	Tier 1 — begin relationship at E1 start
My Aged Care (MAC) / Services Australia [SC][NE]	Switching Cost + Network Effect (government data integration)	4	3	7	My Aged Care is the government portal through which all CHSP and Home Care Package (HCP Level 1–4) clients are registered. Every agency client has a MAC reference number. A MAC API integration would: (a) pre-populate client records (name, package level, care needs assessment), reducing SPP cold-start time, (b) surface vacancy events against MAC-registered visit schedules, and (c) provide a government-validated client identity anchor that deepens the SPP moat. Services Australia controls API access — this is a government partnership, not a commercial one. Access may require a pilot program application.	High — government API access; data governance agreement with Services Australia; 12–18 months	Tier 1 ■ — highest strategic score (7/10); initiate feasibility inquiry NOW, build over 12–18 months
NDIS Quality and Safeguards Commission [SC]	Switching Cost (regulatory alignment for NDIS agencies)	2	2	4	The NDIS QSC oversees quality and safeguards for NDIS-registered providers. Independent NDIS agencies are the O6 greenfield pivot path (new agencies with no existing EMR, no VI-1 barrier). An NDIS QSC alignment document (demonstrating that Home-Care-AI supports compliance with NDIS Practice Standards) de-risks the product for this segment. Requires product to handle NDIS-specific data fields (support coordination, plan management) — scope expansion from aged care.	Medium — alignment document; no commercial terms; requires product extension for NDIS fields	Tier 2 — conditional on O6 pivot path activation
Bupa Australia / Medibank (private health insurers with home care benefits) [NE]	Network Effect (member distribution)	2	2	4	Both Bupa and Medibank offer limited home care benefits under their health insurance products. A product endorsement or preferred provider arrangement could provide distribution access to their home care benefit members. Smaller reach than MAC (government-funded home care dwarfs private), but commercially valuable for premium-tier clients.	Medium — commercial negotiation; product must meet insurer's compliance requirements	Tier 3

Partner	Moat Type	S C	N E	Total	Strategic Rationale	Complexity	Timeline
Mable / Careseekers (carer supply) [NE]	Network Effect — see Integration section	2	3	5	Dual-listed as both integration and distribution partner. As a distribution channel, Mable's agency-facing product (Mable for Business) could be a referral pathway for the product — agencies already using Mable for carer sourcing are experiencing F-1 (availability uncertainty), which is exactly the problem Home-Care-AI addresses for the coordinator. Referral arrangement rather than API integration in the first instance.	Low-Medium — commercial referral arrangement, no API required	Tier 2

Moat Summary for Positioning (Artifact 19 Direct Feed)

Moat Type	Confirmed Partners	Score	Status for Positioning
[SC] Switching Cost	AWS ap-southeast-2 (audit log + SPP storage), MessageMedia (channel layer)	3–4/5	■ Confirmed (infrastructure level only)
[SC] Switching Cost	AlayaCare API integration	4/5	■ Unvalidated — Tier 2; cap at 3/5 for positioning
[SC] Switching Cost	My Aged Care (MAC) API integration	4/5	■ Unvalidated — Tier 1 aspirational; cap at 3/5 for positioning
[NE] Network Effect	ACCPA endorsement + member distribution	5/5	■ Unvalidated — relationship not yet initiated; cap at 3/5 for positioning
[NE] Network Effect	SPP data gravity (internal — not partner-derived)	5/5	■ Confirmed — structural product moat; no partner required
[CV] Clinical Validation	No partner confirmed yet	—	■ Unvalidated — no validation evidence exists; not usable in positioning

Honest moat position at current stage

- **Confirmed, usable in positioning:** SPP data gravity (internal), compliance architecture (internal), AWS infrastructure switching cost (weak but real)
- **Aspirational, flagged ■ Unvalidated:** AlayaCare SC, MAC SC+NE, ACCPA NE, all clinical validation
- **Implication for Artifact 19:** Positioning statement must be grounded in product differentiation (SPP, E-3 gate, privacy-by-design), not in partnership-derived moats that do not yet exist.

3. Value Proposition per Partner Type

For Integration Partners (AlayaCare — Primary Target)

"Integrating Home-Care-AI into AlayaCare means care coordinators fill every vacant visit using trust-weighted matching without leaving their existing scheduling workflow. Agencies that currently use AlayaCare's Vacant Visit Agent — which matches on availability and qualifications only — get a trust layer and a client notification chain that AlayaCare does not offer natively. The integration makes Home-Care-AI the matching intelligence behind AlayaCare's scheduling engine, not a replacement for it."

The VI-1 unlock this creates: Agency owners currently resistant to "another tool" have no objection when Home-Care-AI surfaces within their existing AlayaCare workflow. VI-1 (standalone willingness to pay) becomes less critical if the product lives inside the coordinator's current environment.

For Validation Partners (ACCPA — Primary Target)

"A joint case study with ACCPA, documenting vacancy fill time, cancellation rate reduction, and coordinator experience improvement across two validated agencies, gives every independent home care agency owner in Australia peer evidence from organisations like theirs — not from a vendor white paper. ACCPA's endorsement converts coordinator word-of-mouth into an industry-level trust signal at the moment of agency owner evaluation."

What ACCPA gets: Content for the Future of Aged Care conference; a technology innovation narrative for members; a demonstration that the association is actively shaping AI adoption in the sector (O4 — AI regulatory tailwind).

For Distribution Partners (My Aged Care / Services Australia — Strategic Target)

"An API integration between Home-Care-AI and the My Aged Care portal means that when a registered CHSP or HCP-funded client has a vacant visit, the coordinator's match engine is working from a government-validated client profile — not from a manual spreadsheet entry. For Services Australia, this is a demonstration that approved technology providers can improve visit completion rates for government-funded home care packages. For Home-Care-AI, MAC integration is the deepest switching cost moat in the product: agency data is anchored to government client records, making migration to a competitor a multi-system divorce."

4. Top 3 Moat-Building Partnerships — Deep Dive

Deep Dive 1 — ACCPA (Aged & Community Care Providers Association) [NE][CV]

Moat-Builder Score: 6/10

Partner overview: ACCPA was formed in 2022 from the merger of ACSA (Aged Care Services Australia) and LASA (Leading Age Services Australia). It is now the single peak body representing 1,000+ aged care and community care organisations across Australia — covering the entire independent home care TAM and then some. ACCPA runs the Future of Aged Care national conference, manages member communications, administers innovation awards, and advises the Department of Health and Aged Care on sector policy.

Decision-maker profile

- CEO level for endorsement and conference partnership
- Innovation / Technology Lead for pilot introduction
- State chapter leads for grassroots coordinator engagement
- Entry path: ACCPA runs an "Innovation in Care" awards programme — a submission is a low-cost, high-visibility first contact.

Partnership structure — three stages

Stage	Action	Timing	Cost
Stage 1 — Advisory	Recruit 1 ACCPA-connected clinical advisor to Home-Care-AI advisory board. Offers: early access, named credit, modest equity/retainer.	Now — before E1	Near zero
Stage 2 — Content	Submit E1 case study (Angela + Tom outcomes) to ACCPA's Innovation Awards or as a conference session abstract.	Post-E1 validation (Q2)	Conference fee only
Stage 3 — Endorsement	Formal ACCPA member communication featuring Home-Care-AI as a recommended coordination tool for independent agencies. Requires demonstrated outcomes + ACCPA compliance review.	Post-beachhead validation (Q3–Q4)	Commercial arrangement TBD

Switching cost / network effect mechanism:

ACCPA endorsement creates a trust-based NE: the more agencies adopt because of ACCPA's recommendation, the stronger the association's own reputation as a technology curator — reinforcing both ACCPA's value to members and Home-Care-AI's legitimacy. The network effect is mutual.

Risk:

ACCPA simultaneously represents AlayaCare's existing customer base. If AlayaCare lobbies ACCPA for exclusivity or fast-follows with a competing product, ACCPA may not endorse a challenger. Mitigation: position Home-Care-AI as an AlayaCare complement (matching intelligence layer), not a replacement. This is strategically true in v1 (no EMR, no billing) and reduces the threat perception.

First step:

Search ACCPA member directory for clinical advisors at independent agencies in Victoria or NSW. Warm introduction via Angela (CC-001, CareBridge) or Tom (CC-002). Angela's agency is likely already an ACCPA member — she is the entry path.

Deep Dive 2 — My Aged Care (MAC) / Services Australia [SC][NE]

Moat-Builder Score: 7/10

Partner overview: My Aged Care is the Australian Government's entry point for all government-funded home care. Every client receiving Commonwealth Home Support Programme (CHSP) or Home Care Package (HCP Level 1–4) services has a MAC client record, a unique support plan, and a registered provider assigned to them. Services Australia administers MAC and controls API access. The Department of Health and Aged Care sets the policy framework.

In FY2023–24, there were approximately 900,000 people receiving CHSP services and 200,000+ receiving HCP packages. These are the clients of the agencies we serve.

Decision-maker profile

- Services Australia Digital Partnerships team (API access)
- Department of Health and Aged Care — Aged Care Technology team (policy alignment)
- Entry path: Services Australia's Aged Care Gateway Connectivity Guide; Digital Health CRC for research-backed pilot applications

Partnership structure — feasibility first

This is a 12–18 month strategic initiative, not a Q1 task. The sequence:

Phase	Action	Timing
Feasibility	Request MAC API documentation from Services Australia. Identify which fields are accessible to registered providers: client ID, package level, care needs assessment summary.	Q2 — immediately after E1
Pilot application	Apply to Services Australia Digital Partnerships for a sandbox access agreement. Position as: "improving visit completion rates for government-funded home care clients."	Q3
Limited pilot	Deploy with 2–3 agencies (Angela, Tom + one ACCPA-referred agency). Measure: visits filled per MAC-registered client; cancellation rate; notification compliance.	Q4–Q1 Y2
Full integration	MAC client data pre-populates SPP at onboarding. Vacancy event triggers against MAC visit schedule. Government-validated client anchor deepens SPP moat.	Year 2

Switching cost / network effect mechanism:

Once an agency's client SPP records are anchored to their MAC client IDs, switching to a competing tool means re-mapping the preference graph to a new system — and losing the MAC integration. The switching cost is compound: SPP data + MAC linkage + institutional memory. The NE component: as more agencies use the MAC integration, Services Australia has more evidence of value, making the API partnership more durable and harder for a competitor to replicate quickly.

Risk:

Government APIs are notoriously slow to open and can be unilaterally revoked or changed. Policy changes to the Aged Care Act (ongoing reform) could restructure MAC access rights. Mitigation: build the product so it delivers full value without MAC integration (SPP is the core moat), and treat MAC as a moat-deepener, not a dependency.

First step:

Download the MAC Provider Digital Access (PDA) documentation. Determine whether provider-level API access allows reading of care needs assessment data or only administrative data. This is a desk research task — 2 days.

Deep Dive 3 — AlayaCare (EMR API Integration) [SC]

Moat-Builder Score: 5/10 — strategically critical despite moderate score

Partner overview: AlayaCare is the dominant EMR in Australian home care (estimated 30–40% market share among independent agencies). Founded in Canada, with a strong AU presence. Key features: rostering, care planning, clinical documentation, billing, and — directly relevant — a Vacant Visit Agent that matches on availability and qualifications. AlayaCare's Vacant Visit Agent is the product's primary direct competitor for the automated matching function.

Decision-maker profile

- Partnership / Business Development team (AlayaCare Australia) for commercial integration discussion
- Product team for API access and App Marketplace (AlayaCare Connect) listing
- Entry path: AlayaCare Connect developer program; approach as complementary intelligence layer, not a competitive threat

The VI-1 tension:

Artifact 12 W1 / Artifact 8 identifies VI-1 (standalone willingness to pay, without EMR integration) as a critical assumption under test via XP-2A. If XP-2A returns no LOIs from established agencies, VI-1 fails — which means agencies will not pay for a standalone tool, and the AlayaCare integration becomes a requirement, not a nice-to-have. If XP-2A returns LOIs, standalone value is confirmed and the AlayaCare integration is an accelerant rather than a prerequisite.

Sequencing implication:

Do not approach AlayaCare until XP-2A data is in hand. Approaching as a supplicant needing their distribution is a weaker position than approaching as a validated product seeking to extend its reach through their marketplace.

Partnership structure — two scenarios

Scenario	Condition	AlayaCare Stance	Action
Complementary integration	VI-1 confirmed value (standalone proven)	AlayaCare lists Home-Care-AI in AlayaCare Connect as a "trust matching add-on." Coordinators access via SSO inside AlayaCare.	Approach as: "We add the trust layer your Vacant Visit Agent doesn't have. Combined, you offer care quality matching, not just availability matching."
Competitive threat	AlayaCare launches a trust/preference matching feature	AlayaCare rejects integration; may attempt to block	Accelerate ACCPA endorsement and standalone adoption. The 12-month window (Artifact 13 T1) is the build-moat period. If AlayaCare ships a competing feature, the moat must already be built.

Switching cost mechanism: If coordinators are trained on Home-Care-AI's matching interface within AlayaCare's environment, replacing Home-Care-AI means retraining on a new interface within a familiar system — SC is moderate (3–4/5) but real, especially for agencies with multiple coordinators who have built SPP data.

Risk — the highest single partnership risk in this map:

AlayaCare could: (a) refuse integration and block access, (b) acquire a competitor to add trust matching natively, or (c) copy the SPP feature with their existing data moat. The 12-month window is the critical variable. AlayaCare's product velocity in the AU market and their next conference announcements are the key signals to monitor.

First step:

Register as an AlayaCare Connect developer. Review their API documentation to understand what data is accessible (roster, client records, visit schedules). Confirm whether read-only access to existing vacancy data is available without a commercial agreement. This informs VI-1: if the API is open, integration is feasible; if it is locked, standalone value must be confirmed first.

API Capability Investigation — required before scoping the integration

The artifact used "FHIR/HL7" language inherited from the skill template. In Australian home care, AlayaCare uses a **proprietary REST API (AlayaCare Connect)**, not a standardised FHIR interface. Two integration

models are possible, and only one delivers the "single workflow" value proposition:

Integration model	What the coordinator experiences	SC score	Prerequisite
Read-only	Vacancy data flows in → Home-Care-AI computes match → coordinator approves in Home-Care-AI UI → coordinator manually updates AlayaCare roster	2–3/5	AlayaCare Connect read endpoints only
Bi-directional	Vacancy detected in AlayaCare → match runs in Home-Care-AI → coordinator approves within AlayaCare workflow (ACT-A-02 / 3-Tap Flow surfaced inside AlayaCare) → confirmed assignment written back to AlayaCare roster automatically	4–5/5	AlayaCare Connect write-back endpoints for assignment confirmation

The moat score of 5/10 and the positioning claim ("coordinators never leave their existing workflow") assume **bi-directional** integration. If AlayaCare Connect only supports read, the SC score drops to 3/10 and the "kills the another app objection" claim does not hold. This must be confirmed before any commercial partnership discussion or integration scoping begins. (*Action: AX-02 — see §7.*)

AlayaCare Contingency Map

Three scenarios, three responses — so the product team is never reactive:

Scenario	Signal	Response
AlayaCare neutral / open	API write-back confirmed; App Marketplace listing approved	Proceed with bi-directional integration (Tier 2 timeline). Position as: "trust matching intelligence layer behind AlayaCare's scheduler."
AlayaCare passive denial	API read-only; write-back blocked; App Marketplace listing rejected or ignored	Standalone route only (VI-1 confirmed via XP-2A). ACCPA endorsement is primary distribution. AlayaCare integration deprioritised. ShiftCare integration activated as the EMR integration path for the beachhead.
AlayaCare active threat	AlayaCare announces trust/preference matching feature; blocks third-party integration via T&C change	Accelerate ACCPA endorsement and SPP moat deepening within the 12-month window (Artifact 13 T1). Redirect agencies already on AlayaCare to the standalone product or ShiftCare migration. The data moat (years of SPP preference history) is the defence — a copied feature without historical preference data is a weaker product.

ShiftCare as the parallel Tier 1 track: ShiftCare serves agencies in the 5–50 client range, has a simpler REST API, and has zero competitive incentive to block integration (no Vacant Visit Agent). If AlayaCare denies or delays, ShiftCare is the integration target — not a fallback of last resort, a prepared parallel track.

5. Partnership Risks & Red Flags

Risk	Description	Severity	Mitigation
AlayaCare exclusivity / competitive response	AlayaCare adds trust-matching natively and blocks third-party integration. The 12-month window closes.	■ HIGH	Build SPP moat depth and ACCPA endorsement within 12 months. AlayaCare cannot replicate 2+ years of structured preference data even with a copied feature. Data gravity is the defence.
VI-1 failure — standalone WTP not confirmed	Agency owners will not pay for a tool that doesn't integrate with their existing EMR. XP-2A returns zero LOIs.	■ HIGH	Activate O6 pivot path (greenfield NDIS agencies with no EMR dependency). AlayaCare integration becomes a parallel workstream, not a blocker.
MAC API access denied or too slow	Services Australia does not grant API access, or the process takes 24+ months. The highest-scoring partnership is unreachable in the build timeline.	■ MEDIUM	MAC integration is a moat-deepener, not a product dependency. Full value is delivered without it. Begin the process now; do not wait for it to build v1.
APP 8 — Google Maps	SC-07 (Artifact 10) — Google's servers are US-hosted. Proximity computation transmits carer postcode. If the DPA review (SC-07) finds Google Maps non-compliant, the proximity feature must use an AU-hosted geocoding library.	■ MEDIUM	SC-07 is a low-effort legal review (Artifact 17 AX-01 equivalent). If Google fails: switch to a local geocoding library (OpenStreetMap AU, postcodes.io). Postcode → suburb lookup does not require Google's accuracy or freshness.
ACCPA-AlayaCare proximity	ACCPA represents AlayaCare's existing customer base. ACCPA may be cautious about endorsing a tool that could challenge one of their major vendor sponsors.	■ MEDIUM	Frame Home-Care-AI as an AlayaCare complement. Ensure ACCPA sees the partnership as extending member value, not competing with an existing vendor relationship.
Single infrastructure provider dependency (AWS)	All infrastructure (compute, storage, audit log) lives in AWS ap-southeast-2. An AWS outage or pricing change creates operational and commercial risk.	■ MEDIUM	CloudTrail and S3 Object Lock are AWS-proprietary — the audit log is structurally dependent on AWS. Design the SPP and matching engine to be cloud-portable (DynamoDB → any document store). Audit log dependency on AWS is acceptable given 7-year retention requirements.
Data ownership in co-development agreements	If a validation partner (Bolton Clarke, NARI) provides patient data for research, they may claim co-ownership of model insights or SPP data enhancements derived from that data.	■ MEDIUM	All co-development agreements must explicitly state: (a) no transfer of SPP schema or matching algorithm IP, (b) research data is de-identified and cannot be re-identified using Home-Care-AI's systems, (c) publication rights are joint; model IP rights are Home-Care-AI's exclusively. Legal review of all research agreements before signing.
Carer marketplace (Mable) data governance complexity	Mable's carers are independent contractors with different APP consent models to agency employees. Integrating Mable carer availability data into the matching engine requires a separate consent and disclosure architecture.	■ MEDIUM	Do not integrate Mable until the agency-employee carer model is fully validated and the SPP data model is stable. Tier 3 — Year 2+.

6. Handshake Table — HS-STRAT-05

Per CLAUDE.md Article IV Rule 5: positioning-statement must read this map before running. Moat claims without confirmed partners are capped at 3/5 and flagged ■ Unvalidated.

ID	From (This Artifact)	To (Execution Plugin)	What Must Transfer
HS-STRAT-05a	§2 Moat Summary for Positioning	positioning-statement (Artifact 19)	Confirmed moats (SPP internal, AWS SC) vs. aspirational moats (AlayaCare SC ■, MAC SC+NE ■, ACCPA NE ■). Positioning must be grounded in confirmed moats only. Aspirational moats capped at 3/5 with ■ flags.
HS-STRAT-05b	§4 AlayaCare deep dive — VI-1 sequencing	create-prd (Execution Plugin)	CLAUDE.md Rule 8: no user story is "Ready for Dev" if it depends on AlayaCare integration until XP-2A validates VI-1. Integration stories are backlog items, not sprint items, until VI-1 is confirmed.
HS-STRAT-05c	§4 MAC deep dive — initiate NOW	Action Item	PM Lead to download MAC Provider Digital Access documentation and confirm API field access within Q2. MAC integration is the highest-scoring strategic opportunity (7/10) and a 12–18 month initiative — the clock starts now.
HS-STRAT-05d	§4 ACCPA deep dive — Angela as entry path	Action Item	PM Lead to identify ACCPA membership status of CareBridge (Angela CC-001) and request warm introduction to ACCPA Innovation or Technology contact. First step: ask Angela directly at E1 week 1.

7. Action Items

Due	Owner	Action	Priority
2026-03-28	PM Lead	Contact AlayaCare Connect developer program — request API documentation and App Marketplace listing requirements. Do NOT propose commercial partnership yet — confirm API accessibility first.	HIGH
2026-03-28	PM Lead	Ask Angela (CC-001) whether CareBridge is an ACCPA member and request warm introduction to ACCPA contact. This is the lowest-cost, highest-distribution-leverage step available.	HIGH
2026-03-31	PM Lead	Download MAC Provider Digital Access (PDA) documentation from Services Australia. Determine which client data fields are API-accessible for registered providers.	HIGH
2026-03-31	Legal	SC-07: Review Google Maps Distance Matrix API DPA against APP 8 requirements (carried from Artifact 10). Confirm only carer postcode is transmitted; determine if DPA is sufficient.	MEDIUM
2026-04-07	PM Lead	Research ACCPA Innovation Awards submission timeline. If a call for entries is open, submit E1 case study data (Angela + Tom vacancy fill improvements).	MEDIUM
2026-04-15	PM Lead	After XP-2A results are in hand (Agency Owner LOI experiment): determine whether to approach AlayaCare as a complement (VI-1 confirmed) or accelerate ACCPA/standalone adoption (VI-1 failed). Do not approach AlayaCare before XP-2A data exists.	HIGH — timing-gated
2026-03-28	Engineer	AX-02 — AlayaCare Connect API investigation: confirm whether write-back endpoints exist for assignment confirmation (bi-directional integration). If read-only only: update AlayaCare SC moat score from 5 to 3 in this artifact, and remove the "coordinators never leave their workflow" claim from §3 Value Proposition. Report findings to PM Lead by 2026-04-01.	HIGH

Partnership Investment Summary (PAC Estimates → Artifact 20 inputs)

Rough Partnership Acquisition Cost (PAC) estimates per partner. These are inputs to the AI Unit Economics model (Artifact 20) — not final figures. Nature column distinguishes cash outlay from time/opportunity cost.

Partner	PAC Category	Rough Estimate (AUD)	Nature	Timing
ACCPA Advisory Board seat	Equity + retainer	~0.25% equity + \$2K/year cash	Low cash; equity cost	Now
ACCPA conference / Innovation Awards	Marketing	\$5–15K one-off	Cash — conference sponsorship or submission fee	Post-E1
ACCPA Commercial Endorsement	Membership + marketing contribution	\$20–60K/year recurring	Cash — significant at pre-revenue stage; high distribution ROI	Post-beachhead
AlayaCare Connect integration (read-only)	Engineering time	\$15–25K equivalent (3–5 weeks engineer time)	Time cost	Post-VI-1
AlayaCare Connect integration (bi-directional)	Engineering time + legal (API agreement)	\$30–60K equivalent (6–10 weeks + legal review)	Time + cash	Post-VI-1; subject to AX-02 confirmation
ShiftCare integration	Engineering time	\$10–20K equivalent (2–4 weeks engineer time)	Time cost — simpler API	Tier 1 parallel track

Partner	PAC Category	Rough Estimate (AUD)	Nature	Timing
MAC Government Partnership	PM bandwidth	3–6 months PM time (opportunity cost)	Time cost — competes with build sprints; no cash outlay	Q2 initiation → Year 2 build
Bolton Clarke Pilot	PM time + legal + subsidised access	\$20–50K equivalent	Time + possible revenue deferral during pilot	Post-beachhead validation
NARI Research Partnership	HREC application + PM time	\$5K cash + 18–24 months PM time	Cash + very long horizon; Year 2+ only	Year 2+
Dementia Australia endorsement	Relationship + co-content	\$5–15K equivalent	Mostly time; possible co-publication costs	Tier 2

Total estimated PAC for Tier 1 partnerships (cash only, Year 1): \$30–90K AUD — primarily driven by ACCPA commercial endorsement if pursued at full commercial tier. Advisory board + conference entry route keeps Year 1 PAC under \$20K and delivers the distribution signal needed to validate the endorsement investment.

Full PAC → CAC → LTV model to be built in Artifact 20 (AI Unit Economics).

Partnership moat note: At the current stage (pre-revenue, pre-launch), Home-Care-AI's most durable moat is entirely internal — the SPP data model depth and the compliance-by-design architecture that competitors cannot quickly replicate. No partnership-derived moat is yet confirmed. The positioning statement (Artifact 19) must reflect this honestly: the product earns trust through what it does and how it is built, not through who it is partnered with. Partnerships amplify a moat that already exists — they do not create it from scratch.